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Injectable Weight-Loss Medications in the UK:
A Reflective Essay

Commercial Determinants of Health and Health Policy
Master of Public Health

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Introduction

Commercial determinants of health (CDoH) are strategies and approaches used by the private sector to promote products and choices that are detrimental to health. One such strategy, framing, is a powerful tool used to shape public perception, policy discourse, and regulatory decisions. In the context of CDoH, framing refers to the strategic presentation of information by corporate actors to influence consumers, policymakers, and the scientific community in ways that accommodate their interests, often at the expense of public health (1). This concept is particularly relevant in the case of weight-loss injectables, which have rapidly gained popularity in recent years (2). While clinical trials have demonstrated an average weight-reduction of 15% over 15 years, concerns over dependency, economic burden, and strain on NHS resources raise important public health questions (3–5). As of January 2025, 82 deaths in the UK have been associated with GLP-1RA drugs, including 22 linked to weight-loss use, and severe side-effects such as acute pancreatitis, kidney damage, and gastrointestinal complications have been reported (6). Additionally, the rising demand has contributed to widespread misuse for non-medical purposes and the emergence of a dangerous counterfeit drug market, with 369 fake Ozempic pens seized since 2023 (7).

Despite these risks, regulations remain arguably lax, in part due to the strategic framing of these medications as a necessary solution to obesity, shifting focus away from behavioral and systemic interventions to a medicalized, pharmaceutical-first approach (8). As such, this reflective essay critically explores how framing has influenced public and regulatory discourse surrounding injectable weight-loss medications in the UK. Drawing from theoretical perspectives and empirical literature, it will examine the ways through which marketing, regulatory capture, and agenda-setting strategies have influenced policy decisions. In addition, it will compare these framing strategies to those used in other health-harming industries to highlight common patterns of corporate influence.

The Theoretical Context of Framing in Health Policy

Framing theory, as developed by Goffman in 1974, suggests the way in which information is structured and communicated determines how audiences will perceive and interpret issues (9). In health policy, framing defines the nature of a problem, identifies responsible actors, and prescribes solutions (9). In 1993, Entman further categorized framing into four functions: defining problems, diagnosing causes, making moral judgements, and suggesting remedies (9). Through the CDoH framework in 2016, Kickbusch *et al.* then highlighted how corporations utilize framing strategies to shift attention away from the unfavorable aspects of their products (10). Such groups use tactics such as corporate social responsibility (CSR) campaigns, lobbying, and targeted marketing to build narratives that normalize and sometimes medicalize the use of their products. These

strategies align with Kingdon's multiple streams framework, which explains how problems, policies, and politics must align for changes in policy to occur (11). By influencing these streams, commercial actors shape the policy landscape to maintain a favorable market environment. These strategies have not only been observed in the weight-loss injectable industry, but also others, where harmful products are framed as consumer choices rather than public health concerns (12,13).

Framing in Injectable Weight-Loss Medications

Our policy analysis on injectable weight-loss medications in the UK revealed several instances of strategic framing by pharmaceutical companies such as Novo Nordisk and Eli Lilly, leading manufacturers of semaglutide and tirzepatide-based medications. Injectables, such as Ozempic and Wegovy, were initially developed for diabetes management, but have since been aggressively marketed for the treatment of obesity (14,15). This shift was facilitated by corporate strategies that aimed to frame obesity as a chronic disease requiring lifelong pharmacological intervention (16).

Marketing and the Normalization of Medicalized Weight-Loss

One of the most effective framing methods employed by these companies is the use of indirect marketing. While UK law prohibits direct-to-consumer advertisement for prescription-only medications, this restriction is often circumvented through strategic use of celebrity endorsements and social media influencers – people with normative and relational power (17). Notably, several high-profile figures including Elon Musk, Amy Schumer, and Whoopi Goldberg among others have publicly attributed their weight-loss to semaglutide use, creating consumer demand beyond the originally intended patient population (18). Through inadvertent use of MINDSPACE considerations, they present weight-loss injectables as a lifestyle product rather than a strictly medical treatment, increasing societal acceptance and demand.

Pharmaceutical firms also sponsor medical education programs and continuing professional development (CPD) courses for healthcare providers, reinforcing the narrative that obesity should primarily be managed through pharmaceutical intervention (19,20). This approach aligns with agenda-setting theory, which posits that repeated exposure to specific 'frames' shape both public perception and professional consensus (21). In 2023, Novo Nordisk faced regulatory scrutiny in the UK for its promotional practices, and has recently been found in breach of transparency standards set by the Prescription Medicines Code of Practice Authority (PMCA) for failing to disclose significant payments to healthcare and patient organizations (22). Similar strategies have been employed in the tobacco industry, where industry-sponsored research downplayed the risks of smoking, influencing medical discourse and delaying regulatory action (23).

Regulatory Capture and Supply Manipulation

Another key finding from our policy analysis was the role of regulatory capture in shaping the availability and accessibility of weight-loss injectables. Novo Nordisk was found to have lobbied for expanded access while resisting stricter oversight on off-label prescribing (24). Notably in 2023, the company was suspended from the Association of the British Pharmaceutical Industry (ABPI) for breaching regulations – an indication of its influence in policy negotiations (25).

Yet another framing tactic that benefits corporate interests is supply-demand manipulation. Due to high global demand for semaglutide-based medications, Novo Nordisk has prioritized more profitable markets such as the US, leading to critical shortages around the globe and in the UK (26). This artificial scarcity further reinforces the framing of injectables as high-value and necessary treatments, increasing consumer demand and willingness to pay (27). As elucidated by Kingdon's framework, the problem, policy, and politics streams (being shortages stemming from misuse and high obesity rates, pressure to expand access, and industry lobbying and media attention respectively) intersect to create a policy window favorable to industry interests.

Comparisons with Other Health-Harming Industries

It is worth mentioning that these framing strategies utilized by the weight-loss drug industry are not unique; they have also been well documented in other health-harming industries such as tobacco, alcohol, and ultra-processed foods. For instance, the alcohol industry has long framed alcohol consumption as a matter of individual responsibility rather than a public health issue, thereby resisting stricter regulations (13). In a similar fashion, the tobacco industry strategically framed smoking as a personal choice, downplaying the addictive nature of nicotine and delaying regulatory intervention (12). In the food industry, Nestlé and other major corporations have framed ultra-processed foods as part of a 'balanced diet', rather than contributors to obesity and metabolic diseases (28). Such examples mirror how pharmaceutical companies frame injectables as the ideal solution for obesity, rather than addressing underlying determinants, including dietary habits, socioeconomic factors, and food environments.

Reflections on our Policy Analysis

Conducting our policy analysis provided valuable insights into the subtle yet powerful role of framing in shaping health policy and public perception. One notable challenge we faced was identifying how corporate influence permeated various levels of the policy process, from guideline development to public awareness campaigns. The framing of obesity as a disease requiring long-term pharmaceutical intervention has critical

implications for health equity, as high monthly costs (£160-£300) make these treatments inaccessible to lower-income individuals (29). In addition, illegal sales persist (particularly online and in unauthorized venues such as beauty salons) despite warnings from the Medicines and Healthcare products Regulatory Agency (MRHA), exposing customers to serious health risks (30). Compounding these challenges, UK obesity policies have historically lacked adequate implementation and evaluation plans, often relying on individual behavior change rather than addressing broader societal determinants (31). The resultant policy landscape has created conditions in which weight-loss injectables have been framed as an essential intervention, despite failing to address the root causes of obesity. The contrast between the NHS' tiered eligibility criteria and the widespread availability of these medications through private clinics further underscores the inequitable nature of such commercialized health solutions (8).

Next Steps: Addressing Industry Framing

To mitigate the impact of corporate framing, policy responses must be proactive rather than reactive. Based on our analysis, addressing the industry's prioritization of corporate profits over public health will require changes in current regulation, public messaging, and accountability mechanisms. Recommendations outlined in Table 1 have the potential to counter framing while promoting a more equitable and evidence-based approach to policy. However, it should be considered that political inertia and resource constraints could delay meaningful reform.

Policy	Reasoning	Policy Recommendation Components
Strengthen Regulatory Oversight on Marketing & Prescription Controls	Pharmaceutical companies have capitalized on regulatory gaps, leveraging indirect advertising and physician-targeted marketing to expand their market influence.	Implement stricter advertisement regulations to ensure promotional content provides adequate information on risks, side-effects, and long-term efficacy.
		Enforce harsher penalties for use of misleading or unsubstantiated health claims, including stronger oversight of social media endorsements and influencer partnerships that normalize non-medical misuse.
		Tighten prescription controls, ensuring injectables are prescribed in accordance with strict clinical guidelines rather than being widely available through private clinics and online sources with minimal oversight.
		Enforce mandatory conflict-of-interest disclosures for policymakers and researchers involved in research and guideline development to mitigate industry influence on regulatory decisions.
Shift the Narrative Beyond an Approach Centered Around Pharmac-euticals	Framing of obesity as a disease necessitating pharmaceutical intervention has been dominant in obesity discourse, marginalizing broader structural determinants of health.	Integrate non-pharmaceutical interventions into obesity and weight-loss policy that address underlying determinants, ensuring funding is allocated towards preventative measures including improved access to good nutrition and community-based workout initiatives.
		Reframe public health messaging to counter the narrative that positions pharmaceuticals as the default solution, instead emphasizing multi-pronged approaches to weight-loss and obesity management.
Enhance Public Account-ability & Policy Evaluation	Effectiveness of UK obesity-related policies have been hindered by inadequate and inconsistent implementation with limited evaluation. Without clear accountability measures, corporate interests can continue to exploit policy gaps.	Mandate independent evaluations of obesity strategies, ensuring interventions (including pharmaceutical solutions) are assessed based on long-term health outcomes rather than short-term weight-loss metrics.
		Establish clear mechanisms for expert and public input in policy development, reducing the biased influence of industry-funded research and lobbying.
		Encourage greater transparency in NICE guideline development to ensure that industry-funded studies do not unduly shape weight-loss and obesity treatment recommendation.

Table 1: *Policy Recommendations Addressing the Influence of Framing by the Weight-Loss Injectable Industry.*

Conclusion

In conclusion, various pervasive framing strategies employed by pharmaceutical corporations within the commercial determinants of health through abuse of economic and structural power have unduly shaped public and policy discourse on weight-loss injectables in the UK. By prioritizing profit over more equitable, evidence-based solutions, these strategies perpetuate health inequities and undermine systemic and meaningful approaches to obesity prevention. Addressing this requires robust regulation, reframing public health narratives, and enhancing accountability to ensure public health imperatives take precedence over corporate interests.

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